

Qualitative interview guide – Public representatives

Introduction

Thank you for taking the time to speak with us. We are trying to understand how care for people with serious illness in the ICU can better support patients and their families. We are interested in your views and experiences. There are no right or wrong answers, and you may skip any question if you prefer.

Section A: Understanding serious illness and ICU care

- a) What do you think is most important for the person, when they know they are terminally ill?
- b) What do you think is most important for the family when they know they are caring for a terminally ill person?
- 1. When you think about ICU care for someone with a very serious illness, without much scope of recovery, what do you think is most important?
(Recovery? Comfort? Family support? Clear information?)
- 2. From your experience or observation, what are the biggest difficulties families face during an ICU stay of their relative with a very serious illness, without much scope of recovery?
(Emotional stress? Financial concerns? Lack of information? Uncertainty?)
 - a) Besides medical treatment, what kinds of support do families of patients with terminal illness need, during and beyond the ICU stay? *** (Emotional support? Financial guidance? Spiritual support? Help understanding the system?)
- 3. How are financial resources of the family impacted – due to delay in communicating terminality of their relative admitted to an ICU?

Section B: Communication and decision-making

- 4. How should ICU doctors and healthcare teams communicate with families, when a patient is very sick, to the point of no return?
 - b) How do you think doctors and healthcare teams at OPD or in-patient setting should communicate with families, when a patient is very sick, to the point of no return?
(Honesty? Regular updates? Simple language? Compassion?)
- 5. Who all do you think should be involved during crucial treatment decisions?
(The patient? Family? Doctors? Everyone together?)

Section C: Care planning and support

- 6. Do you think it is important for families to know before-hand, what a person would have wanted in situations when they become too sick to make decisions? Why/ why not?
- 7. Do you think having these discussions about the patient's condition and wishes earlier would help families? Why or why not?
 - a) If yes - How do you propose this may be implemented in the healthcare settings / communities?
 - b) *Do you feel people are open to such a conversation? Do families usually have these conversations?*
 - c) *What makes these conversations difficult?*

Section D: Views on the MCACP / MATHRU approach

MCACP / MATHRU aims to combine good medical care with clear communication, family involvement, timely information sharing, logistic and emotional support for a terminally ill person reaching ICU. The focus is to ensure comfort and quality of life.

8. What are your thoughts about this approach?
(What do you like about it? Is anything difficult to accept?)
9. From your perspective, are there underlying ethical / moral concerns around this approach?
Can you elaborate?

Section E: Closing

10. In your opinion, what does good care for a terminally ill patient and their family look like?
10. Is there anything else you would like to share?